

X12 CONTACT REDUCTION | O (MAX : 2 PT)

Intent : Implement strategies to reduce human contact with respiratory particles and surfaces that may carry pathogens.

Summary : This WELL feature requires projects to implement design and policy strategies to minimize some instances of contact with contaminated respiratory particles, as well as reduce the number of surfaces that are necessary to touch.

Issue : Many viral diseases, including COVID-19¹ and influenza,² are spread by oral or respiratory emissions of liquid particles emitted by an infected person when they cough, sneeze or even exhale.³ Factors that may affect exposure include the size distribution of the respiratory particles,² humidity,^{4,5} air flow^{5,6} and air treatment.^{5,6} While the relative influence of these factors is variable, direct exposure to particles shed by an infected individual may increase a person's odds of acquiring certain diseases.^{1,7} Respiratory and fecal particles can reach surfaces either by direct deposition from the source (e.g., coughing on a surface) or indirectly through hands. On inanimate contaminated surfaces (called fomites), pathogens may survive for a number of hours or even days.^{8,9} Pathogens may spread from these surfaces when touched, potentially infecting people through oral or nasal exposure.¹⁰ Fomites have been linked to disease transmission for many common viral pathogens, including rotaviruses – the most common cause of children's diarrhea in the world – and adenoviruses, and have been associated with some strains of coronaviruses.^{10,11}

Solutions : Implementing design and policy strategies aimed at reducing exposure to some particles shed by infected individuals, like establishing physical distancing among people^{12,13} or providing barriers to prevent respiratory particles,¹⁴ may slow the spread of pathogens.¹⁵ Reimagining spaces to reduce the number of surfaces a person needs to touch, as well as implementing enhanced hygiene protocols for high-touch surfaces, may reduce the risk of pathogen transmission.^{16,17}

PART 1 B REDUCE RESPIRATORY PARTICLE EXPOSURE (MAX : 1 PT)

For All Spaces except Dwelling Units:

1: Contact reduction cues

The following requirements are implemented during periods when higher incidence of respiratory disease is likely:

- a. At least one of the following distancing strategies:
 1. Queuing marks to increase distance between people while waiting in line (e.g., in elevator lobbies, at check-out counters) and while using moving sidewalks and escalators, as applicable.
 2. Screens, protective furnishings or other engineering controls to reduce particle exchange at security check-ins, reception areas, check-out counters and other places with frequent interaction between occupants and a stationary worker.
 3. Self-service systems to control ingress or egress to the project (e.g., at reception desks or checkout counters).
- b. At least one of the following circulation strategies:
 1. One-way hallways and corridors.
 2. Separate entry and exit doors at pedestrian building entrances.
 3. Separate entry and exit for restrooms except single-user bathrooms.

2: Contact reduction policies

The following requirements are implemented during periods when higher incidence of respiratory disease is likely:

- a. All of the following in any shared spaces (e.g., meeting rooms, workspaces, communal kitchens):
 1. Strategies to increase distance among occupants.
 2. Expectations and requirements for usage of face coverings or personal protective equipment.
 3. Clearly communicated rules for occupancy to reduce respiratory particle exposure and rationale for their use.
- b. The project or organization implements at least one of the following communication strategies to educate occupants about the practices implemented by the project to reduce respiratory particle exposure:
 1. Monthly communication (e.g., email, webcast) to all regular occupants.
 2. Prominent signage (physical or digital) at all building entrances and in shared spaces.

Note :

Interiors projects may count base building elevators, entries and exits towards feature requirements, even if outside of the project boundary.

This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

PART 2 ADDRESS SURFACE HAND TOUCH (MAX : 1 PT)

For All Spaces except Dwelling Units & Guest Rooms:

1: Surface touch management

The following requirements are met:

- a. Project offers hands-free operation (through foot, voice, sensor or personal electronic device) or implements other design strategies to avoid hand operation for at least three of the following:
 1. Regularly used pedestrian doors to the project, during regularly occupied hours.

2. Elevators.
3. All water bottle fillers, water faucets, soap and paper towel dispensers.
4. Window blinds and indoor lighting switches and/or controllers.
5. Lids of trash, recycling and reuse bins.
- b. Project supports occupants in maintaining hand hygiene near the following high-touch surfaces:
 1. Handrails, handlebars and other structures that support mobility and accessibility.
 2. Surfaces designed to help individuals with physical and/or visual disabilities fully utilize a space (e.g., push to open door buttons, wheelchair lift controls, tactile maps or signage).
- c. Project establishes and communicates rules and expectations for the usage and cleaning of shared tools and devices (e.g., photocopiers, gym equipment, communal kitchen appliances, utensils) for all regular occupants.

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